

Comprehensive Clinical Cases

Obstetrics and Gynaecology

1. Gynaecology - Case Proforma.

Patient particulars.

Name

Age

Education

Occupation

Date of admission

Date of examination

LMP (date of last menstrual period)

Informant.

Date of admission

Date of examination

Chief complaints :

History of present illness. :

Irregular cycle

Mid cycle bleeding/spotting

Painful menstruation

Excessive flow during menstruation

Passage of clots.

Increased duration of menstrual blood

Decreased duration of flow

Scanty menstrual blood flow

Amenorrhea
Pain abdomen
Palpable Mass per abdomen
Mass coming out of vagina/prolapse
Excessive white discharge
Foul smelling discharge
Pruritis
Lower back ache
Fatigue
Dizziness
Painful coitus
Inability to conceive (rule out all the causes for infertility)
Unusual weight gain.
Symptoms of bladder dysfunction.
Symptoms of bowel dysfunction.
Diagnosed congenital anomalies

Past history.

H/o similar complaints in the past.
H/o Of diabetes, hypertension, asthma, malignancy, Tuberculosis in the past.
H/o surgery and medical interventions.
H/o diagnosed thyroid related problems.
H/o diagnosed congenital /chromosomal anomaly.
H/o blood transfusion
H/o Drug allergy

Menstrual history.

Age of attainment of menarche
Regularity cycles, duration of cycle
How many days of flow
Usage of pads/tampons/cloths-how many /day
Associated dysmenorrhea, passage of clots

Marital history and obstetric history.

Married since how long

Consanguinous/non-consanguinous marriage

What is husband's occupation.

Parity index.

Number of children

Gender

Age

Immunized till date.

Any antenatal/natal /postnatal complications.

Family history.

H/o similar complaints in the family

h/o diabetes,hypertension,asthma,malignancy,Tuberculosis in the past.

Personal history.

Vegetarian /non vegetarian

Appatite

Sleep

Bowel and bladder habits

Subactance abuse

H/o allergy to any drug ,food**Environmental history**

Lives in pakka/kchcha house

How many rooms in the house.

Number of people in the house.

Vector breeding areas around house

Areas of water stagnation.

Adequate light and ventilation.

Cooking with LPG/fire wood

Water supply

Drinking water.

Sanitation

Waste disposal

Socioeconomic history

Head of the family

Number of people in the house

Total income of the family

Per capita income

Summary after history.

General physical examination:

Patient is conscious, cooperative, well oriented to Time place and person.

Built(Skeletal frame work and height)

Nourishment(muscle mass/BMI)

Vitals:

Pulse

Blood pressure

Respiration.

Temperature.

Pallor

Icterus,

Cyanosis

Clubbing
Edema
Lymphadenopathy

Head to toe examination

Systemic examination.

Abdomen examination.

Inspection:

Shape of the abdomen.

Distended or not

If distended -uniform or not

Umbilicus-central/everted/inverted

Flanks

Renal angle

Sinus /scar/dilated veins

Hernial orifices

Visible peristalsis

All regions move equally with respiration or not.

Visible lump/mass per abdomen

Palpation

All the inspectory findings should be confirmed

Shape of the abdomen

Symmetry

Local rise of temperature

Any Tenderness

Abdominal girth

Vertical Length (if distended)

Xiphisternum -umbilical distance

Umbilicus-pubic symphysis distance

Guarding/rigidity

Palpable lump

-how many weeks it corresponds to

-border

-consistency-cystic,soft,hard

-getting above the swelling

(describe the lump in terms of

Size,Shape,surface, skin over the lump,consistency,temperature over the lump,fluctuation,surrounding area)

Renal angle- look for tenderness

Organomegaly -Palpation of spleen ,liver,Kidney.

Percussion

Tympanic note heard

Liver dullness

Tidal percussion

Fluid thrill

Shifting dullness

Percussion of spleen

Cardiac dullness

Ascultation

Bowel sounds heard or not

Venous hum

Arterial bruit

Other system.

CVS

RS

CNS

Summary.

Provisional diagnosis

Differential diagnosis

Investigation

Treatment.

2.Antenatal case-Obstetrics

Patient particulars

Name

Age

Education

Occupation

Date of admission

Date of examination

LMP(date of last menstrual period)

EDD(expected date of delivery)

Gestational age.

Parity index.

Informant.

Date of admission

Date of examination

Chief complaints

History of present illness

History of present pregnancy.

1st trimester:

How many days of amenorrhea

When was the pregnancy confirmed

Urine pregnancy test done?

Conceived with fertility treatment/not

h/o excessive vomiting,fatigue,fever with rashes,palpable swellings(especially

behind the ear),bleeding per vagina,leaking per vagina,pain abdomen, burning micturition ,increased frequency of micturition, exposure to radiation,regular drug intake
Weight gain.burning micturition ,increased frequency of micturition,fever.
Intake of folic acid tablets.
Regular antenatal visits.
1st trimester scan.

2nd Trimester

Quickening felt at which month.
h/o bleeding per vagina,leaking per vagina,pain abdomen,headache,dizziness,blurring of vision,pedal edema (which doesn't relieve even with rest),excessive vomiting,regular drug intake.
Weight gain.
Intake of calcium and iron tablets.
Regular antenatal visits
2nd Trimester scan.

3rd trimester.

Fetal movements well perceived /not
h/o bleeding per vagina,leaking per vagina,pain abdomen,headache,dizziness,blurring of vision,pedal edema (which doesn't relieve even with rest),burning micturition ,increased frequency of micturition,fever.
Weight gain.
Intake of calcium and iron tablets.
Regular antenatal visits
3rd Trimester scan.

Past obstetric history :

Parity index.
Explain in detail each pregnancy.
Any antenatal/natal/postnatal complications?

Mode of delivery
Place of delivery
Gender of the child.
Birth weight of the child.
Umbilical cord clamped immediately
Passage of urine and meconium.
Any usage of NICU
Yellowish Discolouration of the skin
Fever
Discharge from umbilicus
Respiratory distress.
When was Breast feeding initiated
Breast fed regularly.
Immunized till date
How is the child doing now.

Past history.

H/o Of diabetes,hypertension,asthma,malignancy,Tuberculosis in the past.
H/o surgery and medical interventions.

Menstrual history.

Age of attainment of menarche
Regularity cycles, duration of cycle
How many days of flow
Usage of pads/tampons/cloths-how many /day
Associated dysmenorrhea,passage of clots

Marital history

Married since how long
Consanguinous/non-consanguinous marriage
What is husband's occupation.

Family history.

H/o birth of children with chromosomal anomaly/birth defect
H/o twinning in family
diabetes, hypertension, asthma, malignancy, Tuberculosis in the past.

Personal history.

Vegetarian /non vegetarian
Appetite
Sleep
Bowel and bladder habits
Substance abuse

H/o allergy to any drug ,food

Environmental history

Lives in pakka/kachcha house
How many rooms in the house.
Number of people in the house.
Vector breeding areas around house
Areas of water stagnation.
Adequate light and ventilation.
Cooking with LPG/fire wood
Water supply
Drinking water.
Sanitation
Waste disposal

Socioeconomic history

Head of the family
Number of people in the house
Total income of the family
Per capita income

General physical examination.

Patient is conscious, cooperative, well oriented to Time place and person.

Built

Nourishment

Vitals:

Pulse :

Respiration

Temperature.

Pallor

Icterus,

Cyanosis

Clubbing,

Edema,

Lymphadenopathy

Height

Weight

Head to toe examination:

Systemic examination.

Abdomen examination.

Inspection.

Shape of the abdomen

Abdomen uniformly distended

Movement of regions with respiration .

Umbilicus

Flanks

Renal angle

Sinus ,scar,dilated veins.

Stria gravidarum

Linea nigra

Hernial oifices.

Palpation.

Local rise of temperature and tenderness.

Abdominal girth

Symphysio- fundal height (corresponds to gestational age or not)

Leopold's manoeuvre's .

Fundal grip:

Right lateral grip.

Left lateral grip.

Pelvic grip.

First pelvic grip

Second pelvic grip.

Auscultation.

Fetal heart sound

Summary:

Management.